Dependent Personality Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Dependent personality disorder (DPD) is a long-standing, excessive **need to be taken care of** that leads to submissive, clinging behavior and **fear of separation**, beginning by early adulthood (American Psychiatric Association [APA], 2022). It belongs to **Cluster C** (anxious or fearful).

How common: DPD is one of the least common personality disorders: 0.49% of adults in a large U.S. survey (Grant et al., 2004) and 0.78% across Western community studies (Volkert et al., 2018).

Not harmless. Contrary to clinical lore that dependency is mild and passive, high dependency has been linked to greater risk of physical illness, higher health care use and costs, functional impairment, suicidality, and both being abused by partners and abusing children (Bornstein, 2012).

Counselor framing: the core belief is "I can't cope on my own." Each time someone else decides or rescues, the person feels relief, but confidence shrinks further. Counselors must be careful not to become the next caretaker.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. A pervasive pattern, beginning by early adulthood, with **5 or more** of:

- Difficulty making **everyday decisions** without excessive advice and reassurance.
- Needs others to **assume responsibility** for most major areas of life.
- Difficulty **expressing disagreement** for fear of losing support or approval.
- Difficulty **starting projects or doing things alone** because of lack of self-confidence (not lack of motivation or energy).
- Goes to **excessive lengths to obtain nurturance** and support, even volunteering for unpleasant tasks.
- Feels **uncomfortable or helpless when alone** because of exaggerated fears of being unable to care for self.
- **Urgently seeks another relationship** for care and support when a close relationship ends.
- Unrealistically **preoccupied with fears of being left** to take care of self.

Distinguish from: dependence that is realistic (illness, disability, age), culturally expected deference, borderline personality disorder (reacts to abandonment with rage or emptiness rather than submission), avoidant personality disorder, and agoraphobia (APA, 2022).

When counselors must refer (general clinical consensus): screen for **domestic abuse**; people with high dependency may stay in abusive relationships and have higher rates of victimization (Bornstein, 2012). Refer for depression, suicidal thinking, or substance use, and follow safety planning and reporting duties.

3. Frequently used assessment instruments

These are among the most widely used measures for screening, rating severity, and tracking progress in dependent personality disorder. **Screening tools do not diagnose;** a positive screen calls for a clinical interview against DSM-5-TR criteria. Click an instrument's website to find it.

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
SCID-5-PD (Structured Clinical Interview for DSM-5 Personality Disorders)	Diagnosing all ten DSM-5 personality disorders, with an optional self-report screener.	Semistructured clinician interview	Yes	★ Clinical interviewing skill; training recommended	APA Publishing
PID-5 (Personality Inventory for DSM-5)	Dimensional personality trait profile (negative affect, detachment, antagonism, disinhibition, psychoticism); brief and informant forms.	Self-report; 220-item full or 25-item brief form	No (free from the APA for clinical use)	None beyond general clinical training	APA assessment measures

Instrument	Best used for	Format	Purchase required?	Training or qualification	Where to find it
PAI (Personality Assessment Inventory)	Broad clinical and personality assessment for adults, including scales for borderline and antisocial features.	Self-report, 344 items, about 50 to 60 minutes	Yes	★ Level C	PAR
MCMI-IV (Millon Clinical Multiaxial Inventory-IV)	Adults in treatment; personality patterns alongside clinical syndromes.	Self-report, 195 true/false items, about 25 to 30 minutes	Yes	★ Level C	Pearson
MMPI-3 (Minnesota Multiphasic Personality Inventory-3)	Broad personality and psychopathology assessment, with validity scales for over- and under-reporting.	Self-report, 335 true/false items, about 25 to 50 minutes	Yes	★ Level C	Pearson
PHQ-9 (Patient Health Questionnaire-9)	Screening for co-occurring depression.	Self-report, 9 items, about 2 minutes	No (free)	None beyond general clinical training	PHQ Screeners site

★ **= requires professional qualification to purchase or specialized training.** Publishers sell these tests only to qualified users. At Pearson, **Level B** generally means a master's degree in a related field with formal assessment training, or a health care license; **Level C** means a doctorate in psychology or a related field with formal assessment training, or state licensure in a related field (Pearson qualifications policy). MHS, PAR, and WPS use similar levels. Check each publisher's page before ordering. "Original article" links go to the published study that introduced a free scale that has no official website; the scale can usually be obtained there or from its authors.

4. What works: treatment and the evidence

Bottom line for class: there are no trials of treatment for DPD alone. The best evidence comes from trials of Cluster C personality disorders, in which cognitive, dynamic, and schema therapies all helped.

Evidence overview (honest effect sizes):

- **Cluster C trial:** among 50 patients with avoidant, dependent, or obsessive-compulsive personality disorder, 40 sessions of cognitive therapy or short-term dynamic therapy led to improvement maintained over 2 years; 42% (cognitive) and 54% (dynamic) recovered from symptoms, and about 40% in each group recovered in interpersonal problems and personality functioning (Svartberg et al., 2004).
- **Schema therapy:** in a multicenter trial of 323 patients with Cluster C, paranoid, histrionic, or narcissistic personality disorder, schema therapy (which targets long-standing core beliefs) led to recovery more often than usual care or another psychotherapy (Bamelis et al., 2014).
- **Medication (referral notes):** no medication treats DPD itself; co-occurring depression or anxiety may be treated in the usual ways.

Cognitive work with DPD: how it is used (Svartberg et al., 2004; Bamelis et al., 2014):

- **Set a clear, time-limited frame** and gradually hand responsibility back to the client; resist giving advice that the client can work out.
- **Identify and challenge core beliefs:** "I can't manage life without someone to lean on," "If I disagree, they'll leave," "Being alone means being helpless," "Other people know better than I do."
- **Build competence step by step:** practice small independent decisions, assertive statements, and tasks done alone, and record successes as evidence.

5. Religion, spirituality, and Christian integration: what the evidence shows

Disorder-specific research. We found **no study of religion or spirituality in dependent personality disorder** and no faith-integrated treatment trial (literature search, November 1, 2026).

Related evidence. Across 49 studies, positive religious coping (actively partnering with God, seeking support) went with better adjustment, and negative religious coping with worse (Ano & Vasconcelles, 2005). Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and produced added spiritual benefits (Captari et al., 2018).

Honest summary for teaching:

1. Cognitive, dynamic, and schema therapies for Cluster C disorders are the evidence base. Faith integration supports them; it does not replace them.

2. There is no research on faith and DPD. Healthy dependence on God is not the same as handing every decision to other people, including pastors or spouses.
3. Teachings on submission can be misused to keep dependent people in harmful situations. Screen carefully for abuse and spiritual control.

6. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within cognitive, Cluster C-informed care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for dependency framed as spirituality: "A good Christian never disagrees," "I can't decide anything until my pastor or husband tells me," "God wants me to stay no matter how I'm treated," "Making my own choices is pride" (Bornstein, 2012).
- **Challenge (clinical).** Distinguish trust in God from helplessness, and wise counsel from giving up all judgment. Test small decisions and assertive statements and record the outcomes (Svartberg et al., 2004).
- **Replace (pastoral).** Offer Scripture that balances depending on God with personal responsibility and courage:
  - "Carry one another's burdens, and in this way you will fulfill the law of Christ." (Galatians 6:2) and "For each one should carry his own load." (Galatians 6:5). Shared burdens and personal responsibility belong together.
  - "For God has not given us a spirit of fear, but of power, love, and self-control." (2 Timothy 1:7)
  - "Do not fear, for I am with you; do not be afraid, for I am your God. I will strengthen you; I will surely help you…" (Isaiah 41:10). God's help empowers rather than replaces the person.
  - "Trust in the LORD with all your heart, and lean not on your own understanding; in all your ways acknowledge Him, and He will make your paths straight." (Proverbs 3:5–6). Leaning on God, not on any one person.
- **Faith community (clinical).** Encourage a wide circle of support rather than one rescuer, and roles that build confidence. Watch for leaders or partners who exploit compliance.
- **Ministers:** avoid becoming the decision-maker; encourage the person's own prayerful judgment; screen for abuse, and refer when depression or danger is present.

7. Quick classroom takeaways

- DPD is an excessive need to be cared for, with submission and fear of separation (5 of 8 criteria). It is rare (under 1%) but linked to illness, impairment, abuse, and suicidality.
- No DPD-only trials; cognitive, dynamic, and schema therapies help Cluster C disorders. Hand responsibility back gradually.
- Replace "I can't manage without someone to lean on" with God's strengthening presence and the call to carry one's own load.
- Confirm with a structured interview (SCID-5-PD) and screen for depression and for abuse in close relationships.

Sources note

All 8 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record. Assessment instrument links and purchase and qualification levels were checked on the publishers' and distributors' websites, or on Crossref for original articles (Nov 1, 2026).

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